

NUR258SCE Pain Management Simulation Scenario

(Revised 12/11/2025 CA)

Client: Tom Jefferson

Discipline: Adult Health Nursing II

Expected Simulation Run Time: 5 hours

Location: Joyce Simulation Center

Student Level: Advanced Med-Surg

Location for Reflection: Joyce Briefing Center

File Name: Pneumonia NUR258 Pain
Management. NLN

Brief Description of Client

Hospital Location: Medical/Surgical-Telemetry

Attending Provider/Team: Dr. Michael Smertka

Name: Tom Jefferson

Allergies: NKDA

Language: English

Date of Birth: 2/14/19XX

Age: 55

Code: FULL

Admit Weight: 85 kg

Height: 172 cm

Pronouns: He/Him

Sex: Male

Primary Admitting Medical Diagnosis: Post-op Left Hip ORIF with gamma nail placement, anterior approach.

Past Medical History: hypothyroidism, BPH, GERD, HTN

Instructor Synopsis: Mr. Jefferson arrived at the ED yesterday after a fall. He was brought in by EMS because he couldn't stand after falling off his roof. Radiology confirmed a left femoral neck fracture. He was taken to the OR at 1400 yesterday. He was admitted to the Medical/Surgical-Telemetry Unit post-op for observation. He is scheduled to work with physical therapy this morning. Patient is on a PCA pump with 0.2mg/mL of hydromorphone. In this scenario the students will find that he is awake, but lethargic and has depressed respiratory status due to opioid overdose. They will need to assess the client and administer naloxone when ordered by the physician. Students will also need to administer Enoxaparin and provide patient teaching, specifically side effects and precautions. **This is Post-Op Day 2.**

Simulation Design Template (revised February 2023)

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Simulation Learning Objectives

NUR258 Course Outcomes

1. Use critical thinking, clinical reasoning, and clinical judgement to prioritize comprehensive, individualized, and holistic care of the adult client. (CO1)
2. Select appropriate priorities for nursing assessment and interventions when delivering safe nursing care for adults. (CO2)
3. Deliver evidence-based holistic nursing care for adult clients with health alterations. (CO3)
4. Evaluate outcomes of nursing care of adult clients with health alterations. (CO4)

Simulation Scenario Objectives

1. Use critical thinking to recognize cues for pain management and appropriate opioid use. (CO1)
2. Based on recognized cues, perform appropriate interventions with a client in pain and using opioids. (CO2)
3. Reassess a client after administering opioids. (CO4)

Setting/Environment

Medical/Surgical-Telemetry

Equipment/Supplies for Pain Management: Tom Jefferson

Simulated Client/Manikin(s) Needed: HAL

Recommended Mode for Simulator: preprogramed manual advance

Other Props & Moulage:

Equipment Attached to Manikin/Simulated Client:

- ID band for Tom Jefferson
- Male genitalia
- IV in place and working.
- 500 mL bag of 0.9% NS
- Alaris programed IV pump 500mL at 25mL/hr
- PCA pump with hydromorphone 0.2mg/mL syringe in pump.
- Foley catheter in place secured with STAT lock. Clear yellow urine with ~100mL in bag.

Moulage

- Left Hip incision placed as seen in picture below. Open to air.



Equipment Available in Room:

- Equipment to take vitals: Pulse oximeter, telemetry cables (3 lead), thermometer.
- Ambu Bag
- O2 delivery device: nasal cannula and simple mask.
- Toothette or toothbrush
- Blood collection supplies: vacutainer, gauze, tape, flushes, butterfly syringe, & lab tubes.
- Bedpan/urinal
- Black bin to deposit used supplies.
- Emesis basin
- Suction tubing and suction canister.
- Incentive spirometer
- Grip socks
- Water cup with drinking straw.

Beginning Scenario Vitals Monitor programming:

HR: 98 BP: 140/92 Temp: 36.9
RR: 16 SpO2: 93% RA

All Medications and fluids needed for scenario (and quantity of medication)

IV Meds

- naloxone (Narcan) 0.4 mg/mL IV push PRN for respiratory depression. May repeat up to 3 times at 3-minute intervals. (3 doses)

Subcutaneous Meds

- enoxaparin (Lovenox) 60 mg/0.6 mL Subcutaneous injection every 12 hours daily for blood clot prevention. (1 syringe)

Patient Controlled Analgesia (PCA)

- hydromorphone (1 syringe)
DOSE: 0.2 mg/mL
FREQUENCY: every 10 min PRN
BASAL RATE: none
BOLUS: 0.5 mg once every 2 hours PRN

Green: Need in SimServe

Yellow: in the room, running

Roles

<u>Students</u>	<u>Simulation Staff</u>
Lead Nurse	Nursing Instructor & SOS/Tech Team acting as following: • Physician • Charge Nurse • Pharmacy • Radiology • Cardiac/EKG Tech • Phlebotomy/Lab • Operator • Client

Guidelines/Information Related to Roles

All Students: Employ the TeamSTEPPS Principles of leadership, team structure, communications, situation monitoring & mutual support. Monitor clients' condition and intervene as appropriate.

Lead Nurse- Responsible for PT outcome & team performance. Conduct briefing assign tasks & set common goals & objectives. Client assessment. Direct the team in an efficient & effective manner to improve the client's condition. Communicate with the team and MD.

Pre-briefing/Briefing

Prior to report, participants will need pre-briefing/briefing. During this time, faculty/facilitators should establish a safe container for learning, discuss the fiction contract and confidentiality, and orient participants to the environment, roles, time allotment, and objectives.

For a comprehensive checklist and information on its development, go to <http://www.nln.org/sirc/sirc-resources/sirc-tools-and-tips#simtemplate>.

Student Shift Report: **verbally** given to students by instructor.

(Please do not give the students a physical copy of this.)

Client Name Tom Jefferson	Code Status FULL	Allergies NKDA	DOB 2/14/19XX Age 55	Rm # TBD	MD Michael Smertka (Hospitalist)
Vital Signs HR 89 O2 94% RA BP 135/82 Temp 37.5 C RR 16			Health History • Hypothyroidism • BPH • Hypertension • GERD Home Medications: • Tamsulosin 0.4mg PO daily • Levothyroxine 50mcg PO q AM • Lisinopril 10mg PO q AM • Omeprazole 20mg q PM		
Admitting Medical Diagnosis: Post-Op for ORIF of Left Hip with gamma nail placement, anterior approach History of Present Illness: Client was unable to walk or stand after he fell off his roof yesterday. His wife called EMS and he was brought to the ED. He was taken to surgery by Dr. Jonathan Cracking (orthopedic surgeon) yesterday at 1400 and was admitted to the medical/surgical-telemetry unit post-op. An X-ray was done this morning to verify post-op placement of hardware and to clear client for PT today.					
Neuro A&O x4 Follows commands, WNL			Cardiology NSR, S1 & S2 BP stable WNL		
Respiratory Clear, equal, WNL			GI WNL Active bowel sounds Last BM yesterday		
Skin/Wounds Left Hip incision			GU Foley in place due to BPH concerns 525 ml out during night shift		
Drains/IVs Left Hip incision, open to air, clear/minimal drainage. IV: 18 L forearm. Running PCA & fluids. See orders.			Diagnostics WNL		
Pain Mild to moderate, left hip. 3-5/10			Fall Risk & Weight bearing status High fall risk, get out of bed with 2 persons and front wheel walker. Weight bearing status: toe touch on Left leg		
Psycho/Social Wife's name: Becky			Diet Regular diet		

ATI: INITIAL ORDERS (Everything listed below will already be visible to the student in ATI when the scenario is started.)

<u>Medications Visible Pre-Scenario</u>	<u>Medications</u> • Enoxaparin 60 mg/ 1 ml, subcutaneous injection, every 12 hours daily • Naloxone 0.4 mg IV push PRN for respiratory depression. May repeat up to 3 times at 3-minute intervals. • 0.9% NACL IV fluids, 25 ml/hr. Run continuously while PCA is running. • Hydromorphone DOSE: 0.2 mg FREQUENCY: every 10 min PRN BASAL RATE: none BOLUS: 0.5 mg once every 2 hours PRN <u>Home Medications (listed in ATI under NOTES)</u> • Levothyroxine 50 mcg PO Daily, take on empty stomach. • Tamsulosin 0.4 mg PO Daily • Lisinopril 10mg PO Q AM • Omeprazole 20mg PO Q PM																																													
<u>Orders visible pre-scenario</u>	<u>Nursing/Misc. orders</u> • Vital signs: every 4 hrs. • Continuous cardiac monitoring (telemetry). • Continuous oxygen monitoring while client is using PCA. • Respiratory: supplemental oxygen therapy to keep SPO2 greater than or equal to 92%. • Sequential Compression Device (SCD): SCD on client when client is in bed. • Diet: Regular • Reposition every 2 hours while in bed to prevent skin breakdown.																																													
<u>Labs visible pre-scenario</u>	<u>Admission to ED Labs and Imaging PRIOR to surgery</u> <table><tr><td><u>CBC</u></td><td><u>Result</u></td><td><u>Normal</u></td></tr><tr><td>WBC</td><td>9</td><td>5-10</td></tr><tr><td>Hgb</td><td>17</td><td>14-18</td></tr><tr><td>Hct</td><td>50</td><td>42-52</td></tr><tr><td>Platelets</td><td>352</td><td>150-400</td></tr></table> <u>Chemistry/ Metabolic</u> <table><tr><td>Na</td><td>140</td><td>135-145</td></tr><tr><td>K</td><td>3.8</td><td>3.5-5</td></tr><tr><td>Cl</td><td>100</td><td>98-106</td></tr><tr><td>CO2</td><td>28</td><td>23-29</td></tr><tr><td>BUN</td><td>16</td><td>10-20</td></tr><tr><td>Creatinine</td><td>0.6</td><td>0.6-1.2</td></tr><tr><td>Glucose</td><td>118</td><td>60-100</td></tr><tr><td>Ca</td><td>9.2</td><td>9-10.5</td></tr><tr><td>Phos</td><td>4.0</td><td>3.5-4.5</td></tr><tr><td>Mg</td><td>2.1</td><td>1.3-2.3</td></tr></table> <u>X-Ray</u> Positive for Left femoral neck fracture.	<u>CBC</u>	<u>Result</u>	<u>Normal</u>	WBC	9	5-10	Hgb	17	14-18	Hct	50	42-52	Platelets	352	150-400	Na	140	135-145	K	3.8	3.5-5	Cl	100	98-106	CO2	28	23-29	BUN	16	10-20	Creatinine	0.6	0.6-1.2	Glucose	118	60-100	Ca	9.2	9-10.5	Phos	4.0	3.5-4.5	Mg	2.1	1.3-2.3
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Post-Op Day 1 Labs

<u>CBC</u>	<u>Result</u>	<u>Normal</u>
WBC	15	5-10
Hgb	12	14-18
Hct	39	42-52
Platelets	151	150-400

Chemistry/ Metabolic

Na	141	135-145
K	3.9	3.5-5
Cl	98	98-106
CO2	24	23-29
BUN	18	10-20
Creatinine	0.8	0.6-1.2
Glucose	89	60-100
Ca	7.5	9-10.5
Phos	4.7	3.5-4.5
Mg	1.9	1.3-2.3

X-ray



Post-repair- hardware intact. Ok for PT

ATI: AVAILABLE SCENARIO ORDERS (these will all be available to push out in the scenario)

<u>Medications available to order during scenario.</u>	NONE	
<u>Orders available to order during scenario.</u>	• Stop PCA medication. • Place crash cart near client room until client is stabilized. • Notify provider immediately if 3 doses of naloxone are administered and client does not show improvement.	
<u>Lab results available to push out during scenario.</u>	• CBC (will not result in this scenario)	

Scenario Progression Outline

Client Name: Tom Jefferson

Date of Birth: 2/14/19xx

Allergies: NKDA

MD: Dr. Michael Smertka

*****Instructor needs to start the IV fluids and PCA medication BEFORE beginning the scenario.*****

Phase	Manikin/SP Actions	Expected Nursing Interventions	MD Orders and Student Cues
Phase 1	Vitals HR: 98 BP: 140/92 RR: 16 SpO2: 93% Temp: 36.9 Pain: 8/10 ***NOTE: all systems are WNL, except what is listed below. • Neuro: awake, but sleepy • Pain: 8/10	Prior to calling the MD the students should do the following: ☐ Review lab values. ☐ Obtain vitals (BP, HR, O2, Temp, RR, pain). ☐ Perform assessment. Scenario objective #1: Administer Enoxaparin and perform appropriate teaching.	Student Cues • Student needs to administer Enoxaparin. Can ask if they are going to give him his blood thinner or the medication that prevents blood clots if students do not give this medication. • While the student is administering the medication, state "I'm in a lot of pain, I need my pain medication, I'm going to use my PCA pump." • After client uses PCA pump, he is going to become lethargic and have a decreased O2 and RR. Can trend these down and move to phase II.
See Next Page to Continue			

Phase 2	Vitals HR: 110 BP: 118/62 RR: 4 SpO2: 80% Temp: 36.9 Pain: unable to determine ***NOTE: all systems are WNL, except what is listed below. • Neuro: unresponsive, withdraws from pain • Resp: Blue lips, Lung sounds are clear in all quadrants. • GI/GU: foley in place • Skin: incision is intact, clean, dry, open to air AFTER 1st DOSE OF NALOXONE Vitals HR: 96 BP: 128/86 RR: 16 SpO2: 98% Temp: 36.9 Pain: 8/10 • Neuro: awake, follows commands, anxious • Resp: tachypnea, normal skin color. • Pain: 8/10	Prior to calling the MD the students should do the following: ☐ Review lab values. ☐ Obtain vitals (BP, HR, O2, Temp, RR, pain). ☐ Adjust supplemental oxygen, then AMBU bag as needed. ☐ Perform assessment. ☐ Perform sternal rub. ☐ Form a hypothesis and plan of an SBAR with recommendations prior to calling MD. Scenario objectives: Use critical thinking to recognize cues for pain management and appropriate opioid use. (CO1) ☐ Sedation ☐ Respiratory Rate ☐ Glasgow score ☐ Sternal rub Based on recognized cues, perform appropriate interventions with a client in pain and using opioids. (CO2) ☐ Naloxone ☐ Oxygen ☐ STOP PCA Reassess a client after administering opioids. (CO4) ☐ Level of consciousness ☐ Pain	Student Cues • The client is unresponsive but withdraws from pain if the student performs a sternal rub or pinches fingernails. IMPORTANT! • Students will administer the naloxone 1st time and client will wake up. MD Orders • STOP PCA medication. • Place crash cart near client room until client is stabilized. • Notify provider immediately if 3 doses of naloxone are administered.
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Debriefing/Guided Reflection

Debriefing Phase	Debriefing Questions for Consideration
Reactions/ Defuse	How did you feel throughout the simulation experience?
	Give a brief summary of this client and what happened in the simulation.
	What were the main problems that you identified?
Analysis/ Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.
	What were the key assessments and interventions for this client?
	Discuss how you identified these key assessments and interventions.
	Discuss the information resources you used to assess this client. How did this guide your care planning?
	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?
	Explain the nursing management considerations for this client. Discuss the knowledge guiding your thinking.
	What information and information management tools did you use to monitor this client's outcomes? Explain your thinking.
	How did you communicate with the client?
	What specific issues would you want to take into consideration to provide for this client's unique care needs?
	Discuss the safety issues you considered when implementing care for this client.
	What measures did you implement to ensure safe client care?
	What other members of the care team should you consider important to achieving good care outcomes?
	How would you assess the quality of care provided?
	What could you do to improve the quality of care for this client?
Summary/ Application	If you were able to do this again, how would you handle the situation differently?
	What did you learn from this experience?
	How will you apply what you learned today to your clinical practice?
	Is there anything else you would like to discuss?

PAIN MANAGEMENT Tom Jefferson

Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Scenario Overview	Post-op day 2 left hip ORIF	Recent orthopedic surgery with ongoing pain management and mobility restrictions	Important	Creates risk for pain complications DVT fall injury and opioid-related oversedation	Missed post-op complications or unsafe mobility	Monitor post-op recovery pain safety and mobility	Ongoing reassessment of pain mobility and complications	B	Post-op context shapes priorities		No
Scenario Overview	Primary scenario focus = opioid-induced respiratory depression	This is the core teaching point of the scenario	Immediate	Failure to recognize opioid toxicity is life threatening	Respiratory arrest hypoxia cardiac arrest	Identify sedation respiratory decline and intervene immediately	Reassess RR SpO2 LOC and pain after intervention	A R	This is not just a pain scenario it is a reassessment and rescue scenario	✓	Yes
Baseline / Shift Report	Full code	Indicates resuscitative measures would be initiated if patient arrests	Relevant	Important background but not driving current deterioration	Delay in escalation planning if arrest occurs	Know code status	N/A	S	Background awareness only		No
Baseline / Shift Report	NKDA	No known drug allergy reported	Relevant	Medication safety background	Medication error if assumed incorrectly	Verify before meds	N/A	B	Always verify allergies		No
Baseline / Shift Report	Age 55	Adult post-op patient	Relevant	General context only	Minimal	Routine age-appropriate care	N/A	B	Context only		No
Baseline / Shift Report	Hospitalist Michael Smertka	Primary provider for escalation	Relevant	Needed for communication and orders	Delay in escalation if provider unclear	Know who to call	N/A	R	Communication readiness		No
Baseline / Shift Report	HR 89 baseline	Normal baseline heart rate at report	Relevant	Useful for trend comparison	Miss tachycardic deterioration if baseline unknown	Trend vitals	Ongoing	A	Trend baseline to worsening		No
Baseline / Shift Report	O2 94 percent room air baseline	Borderline but acceptable oxygenation baseline	Important	Important because later oxygenation declines significantly	Delayed recognition of deterioration	Trend oxygenation status	Ongoing pulse ox	A	Know starting point before decline	✓	Yes
Baseline / Shift Report	BP 135/82 baseline	Stable perfusion baseline	Relevant	Useful for comparison during deterioration	Miss hypotensive trend	Trend BP	Ongoing	A	Trend awareness		No
Baseline / Shift Report	RR 16 baseline	Normal baseline respiratory rate	Immediate	Respiratory rate is the most important early indicator in this scenario	Missed early opioid toxicity leading to arrest	Count RR manually and trend closely	Frequent reassessment especially after PCA use	A	Respiratory rate matters more than pulse ox alone	✓	Yes

PAIN MANAGEMENT Tom Jefferson

Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Temp 37.5	No current fever concern	Relevant	Not driving scenario	Minimal	Routine monitoring	N/A	B	Not every value is equally important		No
Baseline / Shift Report	Hypothyroidism	Chronic condition	Relevant	Not affecting acute deterioration	Minimal	Routine	N/A	B	Separate chronic history from acute priorities		No
Baseline / Shift Report	BPH	Reason Foley remains in place	Relevant	Helps explain GU management	Miss context for Foley	Monitor GU status	N/A	B	History supports current lines and drains		No
Baseline / Shift Report	Hypertension	Chronic cardiovascular condition	Relevant	Only matters if hemodynamics change	Minimal	Monitor BP	Trend if needed	B	Baseline vs acute		No
Baseline / Shift Report	GERD	Chronic GI condition	Relevant	Not affecting current scenario	Minimal	Routine	N/A	B	Low-yield background		No
Baseline / Shift Report	Tamsulosin home med	Chronic urinary medication	Relevant	No acute impact in scenario	Minimal	Routine	N/A	B	Medication relevance		No
Baseline / Shift Report	Levothyroxine home med	Chronic thyroid replacement	Relevant	No acute impact in scenario	Minimal	Routine	N/A	B	Medication relevance		No
Baseline / Shift Report	Lisinopril home med	Chronic BP medication	Relevant	May matter if hypotension develops	Potential BP impact if given in unstable patient	Monitor BP if administered	Trend BP	B	Hold parameters and clinical judgment		No
Baseline / Shift Report	Omeprazole home med	GI prophylaxis medication	Relevant	No acute impact in scenario	Minimal	Routine	N/A	B	Medication relevance		No
Baseline / Shift Report	Admitting diagnosis = post-op ORIF left hip	Explains pain mobility restrictions and clot risk	Important	Central context for pain and anticoagulation priorities	Missed postop risks	Monitor pain mobility incision and DVT prevention	Ongoing	B	Connect surgery to current nursing priorities		No
Baseline / Shift Report	Fell from roof yesterday	Trauma mechanism for fracture	Relevant	Background only	Minimal	Routine	N/A	B	Context only		No
Baseline / Shift Report	X-ray cleared hardware for PT	Means activity may begin with restrictions	Important	Impacts mobility planning and safety	Fall or reinjury if restrictions ignored	Reinforce mobility limits and assistance	Assess tolerance	B	Clearance does not remove need for precautions		No
Baseline / Shift Report	A&O x4 baseline	Normal baseline neuro status	Immediate	Needed to recognize later change in LOC	Missed progression from alert to unresponsive	Frequent neuro checks	Compare baseline to later phases	A	Subtle neuro change matters	✓	Yes

PAIN MANAGEMENT Tom Jefferson

Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Follows commands baseline	Baseline responsiveness	Important	Important comparison point for opioid decline	Missed CNS deterioration	Assess responsiveness repeatedly	Trend LOC	A	Know normal before decline	✓	Yes
Baseline / Shift Report	NSR baseline	Normal rhythm baseline	Relevant	Useful for comparison	Minimal	Monitor telemetry	Ongoing	B	Stable now does not mean low risk later		No
Baseline / Shift Report	BP stable baseline	No immediate perfusion concern at start	Relevant	Useful context only	Minimal	Monitor	Ongoing	B	Trend awareness		No
Baseline / Shift Report	Respiratory clear and equal baseline	No lung pathology at start	Important	Helps distinguish opioid toxicity from primary pulmonary process	Delay in recognizing central respiratory depression	Assess breath sounds and RR	Reassess after decline	A	Clear lungs do not rule out respiratory failure	✓	Yes
Baseline / Shift Report	Active bowel sounds	Normal GI function	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority finding		No
Baseline / Shift Report	Last BM yesterday	Normal bowel history	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority finding		No
Baseline / Shift Report	Left hip incision	Surgical site to monitor	Important	Infection and healing surveillance	Miss wound complication	Assess incision	Ongoing	B	Post-op site monitoring		No
Baseline / Shift Report	Foley in place due to BPH	Supports GU management and output tracking	Important	Important for safety and perfusion awareness	Missed retention or inaccurate I&O	Maintain Foley assess output	Ongoing	B	Foley purpose and safety		No
Baseline / Shift Report	525 mL urine out overnight	Indicates urine output and perfusion status	Important	Useful fluid balance clue	Missed poor output or renal concern	Track output	Trend I&O	B	Urine output reflects status		No
Baseline / Shift Report	Left hip incision open to air clear minimal drainage	Expected postop wound appearance	Important	Provides baseline wound assessment	Missed wound change or infection	Assess incision	Ongoing	B	Baseline wound appearance matters		No
Baseline / Shift Report	IV 18 gauge left forearm	Patent IV access available	Important	Needed for naloxone and other rescue meds	Delayed treatment if access not assessed	Verify patency and site	Reassess site and patency	B	Always verify access early		No
Baseline / Shift Report	PCA and fluids running	Continuous opioid and IV support in place	Immediate	PCA is the trigger for deterioration and fluids maintain access	Rapid opioid-related respiratory decline	Assess pump settings verify infusion and monitor patient	Frequent reassessment	A R	Pumps must be checked not assumed	✓	Yes

PAIN MANAGEMENT Tom Jefferson

Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Diagnostics WNL baseline	No other acute abnormal test to explain decline	Relevant	Supports focus on opioid cause	Diagnostic anchoring errors	Keep clinical focus on evolving cues	N/A	B	Do not ignore clinical changes because prior diagnostics were normal		No
Baseline / Shift Report	Pain 3 to 5 of 10 baseline	Mild to moderate postop pain	Important	Drives PCA use and pain requests	Unsafe escalation of opioids without reassessment	Assess pain quality location and sedation level	Reassess pain and sedation after interventions	B	Pain treatment must be balanced with safety	✓	Yes
Baseline / Shift Report	High fall risk	Major safety issue in postop patient with opioids and mobility limits	Important	High chance of injury with ambulation or sedation	Fall reinjury bleeding	Use precautions and assistance	Reassess safety status	B	Opioids increase fall risk	✓	Yes
Baseline / Shift Report	2 person assist	Mobility requires staffing support	Important	Unsafe to mobilize alone	Fall or injury	Assist appropriately	Monitor tolerance	B	Safe staffing and delegation		No
Baseline / Shift Report	Front wheel walker	Needed assistive device	Relevant	Supports mobility plan	Fall if not used properly	Use correctly	Ongoing	B	Mobility equipment matters		No
Baseline / Shift Report	Toe touch left leg	Weight bearing restriction protects repair	Important	Unsafe ambulation can damage repair	Reinjury hardware disruption	Reinforce restriction	Assess adherence	B	Students must include restrictions in planning		No
Baseline / Shift Report	Wife Becky	Support system and teaching resource	Relevant	Psychosocial context only	Minimal	Include as appropriate	N/A	B	Holistic care		No
Baseline / Shift Report	Regular diet	No acute diet restriction	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low-yield finding		No
Pre-Scenario Orders	Enoxaparin 60 mg SQ q12h	DVT prophylaxis in postop patient	Important	Required med and explicit scenario objective	DVT or PE if omitted	Administer correctly and teach patient	Assess injection teaching and bleeding risk	B R	Students often focus on crisis and forget routine but important care	✓	Yes
Pre-Scenario Orders	Naloxone 0.4 mg IV PRN	Rescue medication for opioid-induced respiratory depression	Immediate	Life-saving reversal agent	Continued respiratory failure and arrest	Prepare and administer when indicated	Reassess RR LOC SpO2 pain immediately	A R	Naloxone is rescue not routine	✓	Yes
Pre-Scenario Orders	0.9% NS at 25 mL/hr while PCA running	Maintains IV access and hydration	Important	Supports med delivery and line patency	Loss of IV access or delayed rescue treatment	Verify fluid running	Check IV patency	B	Supportive care matters		No

PAIN MANAGEMENT Tom Jefferson

Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Orders	Hydromorphone PCA settings	Defines opioid exposure and risk	Immediate	Pump settings directly affect sedation risk	Medication error or unrecognized oversedation	Verify dose frequency bolus and basal rate	Reassess after use	A	Pump settings are part of assessment	✓	Yes
Pre-Scenario Orders	Vitals every 4 hours	Standard monitoring order	Important	May be insufficient if patient declines	Delayed recognition of deterioration	Increase monitoring frequency based on status	Trend vital signs	A	Nursing judgment can exceed minimum order	✓	Yes
Pre-Scenario Orders	Continuous telemetry	Allows ongoing rhythm monitoring	Important	Supports monitoring during deterioration	Missed rhythm changes	Apply and monitor telemetry	Ongoing	A	Continuous monitoring does not replace bedside assessment		No
Pre-Scenario Orders	Continuous oxygen monitoring while on PCA	Key safety order for opioid use	Immediate	Designed to catch opioid-related decline	Miss hypoxia or delayed recognition	Ensure monitoring is in place and reading	Continuous pulse ox	A	PCA patients require closer monitoring	✓	Yes
Pre-Scenario Orders	Oxygen to keep SpO2 >= 92%	Clear oxygen target	Immediate	Gives intervention threshold	Hypoxia if not addressed	Apply oxygen promptly	Reassess SpO2 and work of breathing	A	Know target parameters	✓	Yes
Pre-Scenario Orders	SCD while in bed	DVT prevention	Important	Postop clot prevention	DVT or PE	Ensure SCD applied and functioning	Ongoing	B	Prevention matters		No
Pre-Scenario Orders	Reposition every 2 hours	Skin integrity order	Relevant	Standard preventive care	Pressure injury	Reposition patient	Ongoing	B	Basic nursing care still matters		No
Pre-Scenario Labs	Admission WBC 9	Normal baseline WBC	Relevant	Useful for trend comparison	Miss postop trend changes	Trend labs	N/A	B	Trend over time matters		No
Pre-Scenario Labs	Admission Hgb 17	Baseline hemoglobin before surgery	Relevant	Useful comparison point	Miss postop expected change	Trend labs	N/A	B	Baseline comparison		No
Pre-Scenario Labs	Admission Hct 50	Baseline hematocrit before surgery	Relevant	Useful comparison point	Miss postop expected change	Trend labs	N/A	B	Baseline comparison		No
Pre-Scenario Labs	Admission platelets 352	Normal baseline platelets	Relevant	Useful before anticoagulation context	Miss bleeding risk trends	Trend if indicated	N/A	B	Know baseline before postop meds		No
Pre-Scenario Labs	Admission Na 140	Normal sodium	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission K 3.8	Normal potassium	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No

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Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Labs	Admission Cl 100	Normal chloride	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission CO2 28	Normal acid-base marker	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission BUN 16	Normal baseline renal marker	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission creatinine 0.6	Normal renal function baseline	Relevant	Useful for comparison only	Minimal	Routine	N/A	B	Baseline renal function		No
Pre-Scenario Labs	Admission glucose 118	Slightly elevated glucose	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission calcium 9.2	Normal calcium	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission phosphorus 4.0	Normal phosphorus	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Admission magnesium 2.1	Normal magnesium	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Initial X-ray positive left femoral neck fracture	Confirms injury requiring ORIF	Relevant	Background context only	Minimal	Routine	N/A	B	Mechanism and diagnosis connection		No
Pre-Scenario Labs	Post-op day 1 WBC 15	Elevated postop WBC	Relevant	May reflect inflammation or stress response	Over-calling infection in wrong scenario	Monitor trend	N/A	B	Context matters with labs		No
Pre-Scenario Labs	Post-op day 1 Hgb 12	Expected postop drop in hemoglobin	Relevant	Important context but not current crisis	Miss bleeding trend if worsened	Monitor trend	N/A	B	Expected postop changes vs dangerous changes		No
Pre-Scenario Labs	Post-op day 1 Hct 39	Expected postop hematocrit drop	Relevant	Context only	Miss bleeding trend if worsened	Monitor trend	N/A	B	Lab trends not isolated values		No
Pre-Scenario Labs	Post-op day 1 platelets 151	Low-normal platelets	Relevant	Context for anticoagulation	Potential bleeding concern if trending down	Monitor if indicated	N/A	B	Platelet awareness with Lovenox		No
Pre-Scenario Labs	Post-op day 1 Na 141	Normal sodium	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 1 K 3.9	Normal potassium	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 1 Cl 98	Normal chloride	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No

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Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Labs	Post-op day 1 CO2 24	Normal acid-base marker	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 1 BUN 18	Normal renal marker	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 1 creatinine 0.8	Normal renal function	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 1 glucose 89	Normal glucose	Relevant	Not central to scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 1 calcium 7.5	Low calcium	Relevant	Abnormal but not driving scenario	Over-focusing on low-yield abnormality	Note but prioritize bigger issue	N/A	B	Not every abnormal lab is the main problem		Yes
Pre-Scenario Labs	Post-op day 1 phosphorus 4.7	Slightly elevated phosphorus	Relevant	Not central to scenario	Minimal	Note only	N/A	B	Low-yield abnormality		No
Pre-Scenario Labs	Post-op day 1 magnesium 1.9	Low-normal magnesium	Relevant	Not central to scenario	Minimal	Note only	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-repair x-ray hardware intact ok for PT	Confirms repair stable for prescribed mobility	Important	Needed for safe mobility planning	Fall or reinjury if restrictions ignored	Follow PT and mobility restrictions	Assess tolerance	B	Clearance is not unrestricted activity		No
Phase 1 Progression	Pain 8 out of 10	Acute pain increases PCA use	Important	High pain is the trigger for PCA use and later decline	Increased opioid self-administration and oversedation	Assess pain before and after PCA use	Reassess pain and sedation	B A	Pain assessment must include safety	✓	Yes
Phase 1 Progression	Awake but sleepy	Early CNS depression	Immediate	Early cue of opioid effect before severe decline	Missed early deterioration	Frequent neuro and sedation assessment	Trend LOC closely	A	Do not dismiss sleepiness in PCA patient	✓	Yes
Phase 1 Progression	Student should review lab values	Required preparation step before calling provider	Important	Supports informed assessment	Poor clinical reasoning or incomplete SBAR	Review chart before escalation	N/A	R	Preparation matters		No
Phase 1 Progression	Student should obtain full vitals	Core assessment requirement	Immediate	Needed to detect deterioration	Missed changing condition	Obtain and trend full vitals	Repeat after interventions	A	Assessment before escalation	✓	Yes
Phase 1 Progression	Student should perform assessment	Bedside assessment required before escalation	Immediate	Nurse must identify sedation and respiratory risk	Delay in rescue treatment	Perform focused and full assessment	Repeat after changes	A	Assessment drives action	✓	Yes

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Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Phase 1 Progression	Administer Enoxaparin and teaching	Explicit learning objective and routine postop care	Important	High-value care may be missed when distracted by pain	DVT prophylaxis omitted or poor patient education	Administer Lovenox safely and teach patient	Assess understanding and bleeding concerns	B R	Do not miss routine critical care during evolving scenario	✓	Yes
Phase 1 Trigger Event	Patient says he will use PCA	Trigger event for opioid decline	Immediate	Marks beginning of deterioration sequence	Missed cause of decline	Closely monitor after PCA use	Reassess within minutes	A R	Every opioid administration needs reassessment	✓	Yes
Phase 1 Trigger Event	PCA use followed by lethargy	Direct cause-and-effect cue	Immediate	Shows opioid impact on CNS	Progression to unresponsiveness	Stop using comfort-only mindset and reassess immediately	Reassess LOC RR SpO2	A	Link intervention to patient response	✓	Yes
Phase 1 Trigger Event	PCA use followed by decreased O2 and RR	Classic opioid toxicity progression	Immediate	Life-threatening respiratory suppression	Respiratory arrest	Recognize toxicity and intervene	Continuous reassessment	A	This is the key transition in the scenario	✓	Yes
Phase 2 Deterioration	HR 110	Tachycardia with distress and hypoxia	Important	Reflects physiologic compensation	Worsening instability	Trend HR and treat cause	Reassess after rescue	A	Compensation can accompany respiratory failure		No
Phase 2 Deterioration	BP 118/62	Lower than baseline but still perfusing	Important	Shows change from prior state	Miss trend toward instability	Trend BP	Reassess after intervention	A	Trend not just normal range		No
Phase 2 Deterioration	RR 4	Critical respiratory depression	Immediate	Severe hypoventilation is life threatening	Respiratory arrest and cardiac arrest	Support airway breathing stop PCA give naloxone	Ongoing frequent reassessment	A R	Respiratory rate is the key rescue trigger	✓	Yes
Phase 2 Deterioration	SpO2 80 percent	Critical hypoxia	Immediate	Indicates inadequate oxygenation	Hypoxic injury arrest	Apply oxygen support ventilation	Continuous pulse ox	A R	Do not wait for lower values to act	✓	Yes
Phase 2 Deterioration	Pain unable to determine	Patient too sedated to report pain	Important	Reflects severe CNS depression	False assumption that no pain means stable	Prioritize ABCs over pain score	Reassess pain after reversal	B	ABC before comfort		Yes
Phase 2 Deterioration	Unresponsive withdraws from pain	Severe LOC decline	Immediate	Indicates dangerous CNS depression	Airway compromise arrest	Perform sternal rub assess GCS call for help	Reassess LOC after rescue	A R	Unresponsive patient needs immediate escalation	✓	Yes

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Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Phase 2 Deterioration	Blue lips	Cyanosis from hypoxia	Immediate	Late sign of severe oxygen deficit	Impending arrest	Provide oxygen ventilatory support	Reassess color and SpO2	A	Late sign means patient is already in danger	✓	Yes
Phase 2 Deterioration	Lung sounds clear despite respiratory failure	Supports opioid cause rather than fluid overload or bronchospasm	Important	Respiratory failure can be central not pulmonary	Delay in identifying opioid toxicity	Assess full respiratory status not just lungs	Reassess after naloxone	A	Clear lungs do not equal stable breathing	✓	Yes
Phase 2 Deterioration	Foley still in place	GU device remains present but not main issue	Relevant	Context only	Minimal	Maintain device	N/A	B	Do not get distracted from ABC priorities		No
Phase 2 Deterioration	Incision intact clean dry open to air	Wound not causing current deterioration	Relevant	Important only as context	Distraction from actual cause	Note and move on	N/A	B	Not every postop cue is the priority		Yes
Phase 2 Expected Actions	Adjust supplemental oxygen then ambu bag as needed	Primary breathing support action	Immediate	Airway and breathing take priority	Hypoxic arrest	Apply O2 and ventilatory support	Reassess SpO2 RR work of breathing	R A	Escalate oxygen support appropriately	✓	Yes
Phase 2 Expected Actions	Perform sternal rub	Assesses responsiveness in unresponsive patient	Immediate	Determines neurologic responsiveness	Missed LOC severity	Stimulate and assess response	Reassess response	A	Know practical bedside responsiveness checks	✓	Yes
Phase 2 Expected Actions	Form hypothesis and SBAR before calling MD	Promotes organized clinical reasoning	Important	Improves communication and recommendations	Incomplete escalation	Prepare focused SBAR	N/A	S A R	SBAR should be concise and recommendation-based	✓	Yes
Phase 2 Intervention	Naloxone administration	Reverses opioid effect	Immediate	Life-saving rescue medication	Continued respiratory depression and arrest	Administer promptly per order	Reassess RR LOC SpO2 pain immediately	A R	Naloxone is a rescue with expected response	✓	Yes
Phase 2 Intervention	Oxygen	Supports oxygenation during opioid toxicity	Immediate	Addresses ABC priority	Hypoxic injury	Apply and titrate as needed	Reassess SpO2 and color	A R	Support while cause is reversed	✓	Yes
Phase 2 Intervention	STOP PCA medication	Removes source of further opioid exposure	Immediate	Essential to prevent repeat decline	Ongoing oversedation	Stop PCA and verify discontinuation	Reassess status and pump	A R	Must remove the cause not just treat the effect	✓	Yes
Phase 2 Intervention	Crash cart near room until stabilized	Escalation and readiness for arrest	Important	Patient is high risk	Delay in emergency response	Bring crash cart and prepare team	Ongoing	A	Anticipate deterioration		No

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Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Phase 2 Intervention	Notify provider if 3 naloxone doses and no improvement	Escalation threshold	Immediate	Signals failed reversal or worsening condition	Delay in higher level rescue	Call provider immediately if no response	Reassess after each dose	R	Know when bedside care is not enough	✓	Yes
Post-Naloxone Response	HR 96	Improved physiologic status	Important	Shows partial stabilization	Miss improvement or relapse	Trend HR	Ongoing	A	Compare before and after intervention		No
Post-Naloxone Response	BP 128/86	Improved hemodynamic status	Important	Supports stabilization	Miss response trend	Trend BP	Ongoing	A	Assess full response		No
Post-Naloxone Response	RR 16	Respirations returned to baseline	Immediate	Key sign naloxone worked	Miss recurrent respiratory depression	Continue close monitoring	Frequent reassessment	A R	Naloxone effect must be monitored for recurrence	✓	Yes
Post-Naloxone Response	SpO2 98 percent	Improved oxygenation	Immediate	Shows response to rescue	Miss re-sedation or rebound decline	Continue pulse ox and monitor	Continuous reassessment	A R	Improvement does not mean done	✓	Yes
Post-Naloxone Response	Awake follows commands	Improved LOC after naloxone	Immediate	Confirms opioid effect was primary problem	Miss recurring sedation	Continue neuro checks	Frequent reassessment	A R	Return to baseline helps confirm hypothesis	✓	Yes
Post-Naloxone Response	Anxious after naloxone	Expected after reversal and pain return	Important	May occur as patient wakes and pain returns	Misinterpret response as new problem	Support patient explain treatment	Reassess anxiety and pain	B	Prepare students for expected rebound effects		Yes
Post-Naloxone Response	Tachypnea after naloxone	Common rebound respiratory response	Important	Can reflect recovery and anxiety	Mislabeling as worsening without context	Continue monitoring work of breathing	Reassess trend	A	Interpret post-reversal findings in context		Yes
Post-Naloxone Response	Pain 8 out of 10 after reversal	Pain returns once opioid effect reversed	Important	Students must now manage pain safely	Re-dosing opioids unsafely or undertreating pain	Notify provider and rethink pain plan	Reassess pain and sedation	B R	Rescue creates a new pain-management problem	✓	Yes
Scenario-Wide Teaching Point	Reassessment after every opioid administration	Core safety principle	Immediate	This is the major clinical learning point	Failure to catch deterioration early	Always reassess sedation RR SpO2 LOC after opioids	Ongoing	A R	Every intervention requires reassessment	✓	Yes
Scenario-Wide Teaching Point	Balance pain control with patient safety	Core nursing judgment issue	Important	Pain treatment without reassessment can harm patient	Oversedation or undertreatment	Use multimodal thinking and safe escalation	Reassess both pain and sedation	B	Comfort never replaces ABC safety	✓	Yes

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Scenario Overview	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching or Debrief Focus	High Risk	Common Missed Items
Scenario-Wide Teaching Point	Use concise focused SBAR	Communication competency	Important	Needed for rapid escalation	Delayed or poor provider response	Report situation background assessment recommendation	N/A		SBAR is a skill not just a template	✓	Yes

Pain Management

The scenario centers on recognizing opioid oversedation, providing safe opioid management, and reassessing after naloxone administration.

Critical Cues

- ☐ Sedation
- ☐ Respiratory depression
- ☐ Low oxygen saturation
- ☐ PCA use

Expected Priorities

- ☐ Airway
- ☐ Breathing
- ☐ Medication safety

Expected Interventions

- ☐ Stop PCA
- ☐ Administer naloxone
- ☐ Oxygen/ventilatory support
- ☐ Notify provider

Expected Reassessment

- ☐ Respiratory status
- ☐ Level of consciousness
- ☐ Pain

Common Student Errors

- Treating pain first
- Missing respiratory depression
- Forgetting reassessment
- Delayed naloxone

CJMM-ALIGNED SIMULATION DEBRIEFING QUICK REFERENCE GUIDE

This guide provides flexible reflection prompts aligned with the NCSBN Clinical Judgment Measurement Model (CJMM). Select, adapt, and sequence questions based on simulation objectives, student performance, and learner needs. Use the prompts as a cognitive aid to support discussion and reflection, not as a required script or checklist.

1. Recognize Cues

- What patient findings or changes in condition stood out to you?
- Which assessment findings were most important?
- What changes suggested that the patient's condition was improving or worsening?
- Were there any important cues that were initially missed or overlooked?

2. Analyze Cues

- What did the assessment findings suggest was happening with the patient?
- How were the important cues related to one another?
- How did the patient's pathophysiology help explain the findings?
- What additional information would have helped you better understand the patient's condition?

3. Prioritize Hypotheses

- What did you believe was the patient's highest-priority problem?
- Which patient problem or concern required attention first, and why?
- What was the greatest immediate risk to the patient?
- How did you decide which problem was most urgent?

4. Generate Solutions

- What nursing interventions did you consider for the patient's priority problem?
- What outcome did you expect from those interventions?
- What other interventions or approaches could have been considered?
- What additional resources, information, or team members could have helped guide the plan of care?

5. Take Action

- What action did you take first, and why?
- How did the patient's condition influence the order of your interventions?
- Were there any actions that should have been taken earlier or differently?
- How did communication and teamwork affect the care provided?

6. Evaluate Outcomes

- How did you determine whether the patient responded to the interventions?
- What findings needed to be reassessed after the interventions?
- Did the patient's response require a change in the plan of care?
- What would you continue to monitor or reassess if care of this patient continued?

7. Reflection and Future Clinical Application

- What went well, and what challenged you the most?
- What would you do differently if you encountered a similar situation again?
- How will you apply what you learned from this experience to future patient care?